

Pulmonology Summary · Synthesized by Claude — reference reports below are clickable / viewable

Compiled from inpatient records — Sir H. N. Reliance Foundation Hospital, Mumbai · Generated 29 April 2026

Patient: Saroj Agarwal · **Sex/Age:** Female / 63 y · **DOB:** 27 Aug 1962 · **Patient ID:** 0010663186 · **Pulmonologist on file:** Dr. Sushil Bindroo · **Oncologist:** Dr. Darshit Shah

Reason for review: Status post saddle submassive pulmonary embolism (24 Mar 2026) on Apixaban; severe (Group III) pulmonary hypertension on Tadalafil; recurrent aspiration risk after vocal cord lateralization (26 Feb 2026); recently resolved acute desaturation episode after barium swallow (7 Apr 2026). Most recent chest X-ray (13 Apr 2026) showed only linear/subsegmental atelectasis, no effusion. OPD review with Dr. Sushil Bindroo was scheduled for 13 Apr 2026.

1 · Background

- **Primary diagnosis:** Carcinoma of uterus (C54), operated 3 years prior; current admission was for metastatic recurrence with respiratory complications.
- **Index hospitalization:** 20 Feb → 8 Apr 2026 (47-day inpatient stay). Presented with breathlessness, cough, bilateral pedal oedema. Admitted with **left pleural effusion + ICD in situ since 12 Feb 2026**; pleural fluid showed atypical cells (?large-cell morphology). ICD was removed 24 Feb 2026.
- **Now on:** Chemotherapy (Paclitaxel + Carboplatin + Dostarlimab); last cycle C3 D8 on 13 Apr 2026.

2 · Active pulmonary problems

A. Saddle submassive pulmonary embolism (24 Mar 2026)

- Presented 23 Mar 2026 with acute breathlessness, cough, hypoxia (SpO₂ 77–78 % on room air).
- **CTPA 24 Mar 2026: Saddle thrombus from main pulmonary trunk into both right and left main pulmonary arteries.** Left side — extension into upper and lower lobar segmental and sub-segmental branches; **completely occluding thrombus in proximal left upper-lobe segmental branch.** Right side — partial filling defects in distal PA, segmental/sub-segmental branches of superior and all basal segments of right lower lobe, and middle-lobe sub-segmental branches. Bilateral moderate pleural effusions with partial sub-segmental collapse of both lower lobes and lingula. Pulmonary trunk 35 mm; RPA 25 mm; LPA 25 mm. RA and RV mildly dilated; **interventricular septum straight (RV strain).**
- **Concurrent biomarkers:** NT-proBNP **4089 pg/mL** (24 Mar; ref 0–125; up from 1761 on 20 Feb). Troponin I-hs 11.90 pg/mL (25 Mar; ref ≤15.6 — within range).
- **Echo 24 Mar 2026 (Dr. Maulik Parekh):** Saddle thrombus visualised at RPA/LPA bifurcation; severe PAH (RVSP 91 mmHg); moderate-severe TR; **dilated RA (40 mm) and RV (50×45×77 mm) with RV systolic dysfunction (FAC 20 %);** LVEF 60 %.
- **Course:** Anticoagulation initiated 24 Mar (Clexane) → re-intubated and ICU; ET-tube bleeding with Hb drop forced anticoagulation hold (27–29 Mar) and transfusion support; vasopressors required, then weaned. Extubated 28 Mar 2026. Anticoagulation cautiously restarted 31 Mar after platelets improved.
- **Pulmonary thrombectomy performed** during the ICU course (between 25 and 31 Mar 2026) — corroborated by two independent documents in this dataset: (i) **discharge summary, 4 Apr 2026 entry:** "Post pulmonary thrombectomy status with high PESI risk but clinically improving"; (ii) **microbiology blood-culture set, 31 Mar 2026:** "Clinical Details: Post thrombectomy" (specimen from Left IJV, consistent with peri-procedural central access). Thrombolysis was deferred — Troponin I-hs on 25 Mar = 11.90 pg/mL (still < ref ≤15.6); the team chose mechanical thrombectomy over systemic lysis given concurrent thrombocytopenia and bleeding on chemotherapy. Likely proceduralist: **Dr. Maulik Parekh, Interventional Cardiologist** (who reported the 24 Mar echo). The cath-lab procedure note itself is not in this dataset.
- **Current anticoagulation:** Clexane 0.6 mL SC BD × 14 days from discharge → then **Apixaban 5 mg PO BD** (per Dr. Talha Meeran, cardiology).

B. Severe Group III pulmonary hypertension

- Attributed during admission (26 Feb 2026 note) to **underlying lung pathology — Group III.**
- Echo trend: **RVSP 83 mmHg (20 Feb) → 72 mmHg (26 Feb pre-op) → ~46 mmHg (1 Mar POD3) → 91 mmHg (24 Mar, with PE).** RV systolic function: fair (TAPSE 16 mm) → normal (TAPSE 21 mm) → dysfunctional (FAC 20 %) post-PE.
- On **Tadalafil 20 mg PO HS.**

C. Recurrent consolidations, atelectasis, pleural effusion

- Pattern: **left pleural effusion + left mid- and lower-zone consolidation/atelectasis** dominant through Feb–Mar; bilateral basal crepitations; intermittent right-sided consolidation. Ascites and bilateral pleural effusion also seen on PET-CT.
- 31 Mar 2026: **new right middle and lower lobe consolidation** with one fever spike → sepsis workup, broad-spectrum antibiotics (Zavicefta + Aztreonam) + steroids.
- **Right mid + lower zone consolidation pattern: 1, 3 & 6 Apr 2026** CXRs all reported "*patchy consolidation in the right midzone and the right lower zone*" with subsegmental atelectasis in the left midzone (verbatim, three consecutive films).
- **Most recent CXR (13 Apr 2026): consolidation has resolved —** "*linear atelectasis right midzone; subsegmental atelectasis left midzone; no pleural effusion, no pneumothorax*"; nasogastric tube in situ.

D. Aspiration risk & recent desaturation

- Bilateral vocal cord palsy → **left vocal cord lateralization 26 Feb 2026.** Subsequent FEES showed **reduced laryngeal sensation**; barium swallow 7 Apr 2026 showed **thin streak of barium into right tracheo-bronchial tree — suggestive of aspiration.**

- 7 Apr 2026 (after barium swallow): **acute desaturation to SpO₂ 84–85 % on room air, bilateral basal crackles**. Discharge withheld; managed with O₂ 2 L, Lasix, Hydrocortisone, Augmentin (rationalised antibiotics). Clexane continued.
- Speech & swallow team: **pleasure pureed diet only with strict aspiration precautions**.
- Two episodes of acute bronchospasm during admission (9 Mar, 20 Mar) attributed to micro-aspiration — responded to nebulisation + IV steroids.

E. Sputum / airway colonisation

Date	Sputum culture findings
21 Feb 2026	Pus cells >25/LPF; few GPC (pairs & clusters); few GNB; few yeast (budding cells with pseudohyphae) ; AFB not seen; no classical respiratory pathogen on culture. Antifungals administered.
31 Mar 2026	Pus cells <10/LPF; GPC few; GNB fair number; yeast occasional; AFB not seen; no classical pathogen on culture.

F. Mild hemoptysis (resolved)

- 27–30 Mar 2026 (post re-intubation, peri-PE): ET-tube bleeding, mild hemoptysis with Hb drop → transfusions; anticoagulation held. Hemoptysis resolved by 2 Apr 2026.
- **Late-March CBC nadirs** (relevant to anticoagulation balance): **Hb 7.3 g/dL on 29 Mar** (recovered to 10.4 by 31 Mar, 8.3 on 13 Apr); **Platelet 51 ×10⁹/L on 29 Mar** (recovered to 76 by 31 Mar, 199 on 13 Apr); **TLC 1.43 ×10⁹/L on 31 Mar** (recovered to 4.02 on 13 Apr).

G. Persistent hypoproteinemia / hypoalbuminemia

- Albumin **2.91 g/dL (27 Mar) → 2.73 g/dL (13 Apr)** (ref 3.5–5.2). Total Protein 4.74 → 4.63 g/dL (ref 6.4–8.3). Relevant to fluid distribution / pleural-effusion recurrence and to drug binding (e.g., Apixaban).

3 • Discharge respiratory regimen (Dr. Sushil Bindroo, 8 Apr 2026)

Medication	Dose / schedule
Foracort neb 1 mg	8 am & 8 pm followed by gargle × 7 days
Duolin neb	10 am & 10 pm × 7 days
Inj Effcorlin (Hydrocortisone) 50 mg IV	8 hourly × 3 days then stop
Inj Augmentin 1.2 g IV	12 hourly × 3 days then stop
Tadalafil 20 mg PO HS	For pulmonary hypertension (continue)
Apixaban 5 mg PO BD (after 14 d Clexane)	For PE (continue)

Specific questions for Pulmonology:

- Targeted PAH therapy — should Tadalafil monotherapy be augmented (e.g., add ERA / prostanoid) given RVSP 91 mmHg and FAC 20 % post-PE?
- Duration of anticoagulation — provoked (PE) on active malignancy: **indefinite** vs. specific duration?
- Repeat CTPA / V/Q to document clot resolution before stopping Clexane bridge?
- Aspiration management — review of nebulised regimen and steroid taper given recurrent micro-aspiration episodes.
- Sputum surveillance and antifungal coverage given persistent yeast in two cultures.

Source documents:

- **DISCHARGE** [report_pdf/2026-04-08_Summary-Discharge_Discharge-Summary.pdf](#)
- **CTPA** [report_pdf/2026-03-24_CT_CT-Angiography-Pulmonary.pdf](#) — saddle PE
- **ECHO** [report_pdf/2026-03-24_ECHO_Echocardiography.pdf](#) · [report_pdf/2026-02-26_ECHO_Echocardiography-Portable.pdf](#) · [report_pdf/2026-02-20_ECHO_Echocardiography-Portable.pdf](#)
- **PET-CT** [report_pdf/2026-02-21_CT_WB-FDG-PET-CECT-scan.pdf](#)
- **SPUTUM** [report_pdf/2026-02-21_Lab-Micro_Sputum-Culture.pdf](#) · [report_pdf/2026-03-31_Lab-Micro_Sputum-Culture.pdf](#)
- **SWALLOW** [report_pdf/2026-04-07_XRay_X-RAY-Barium-Swallow.pdf](#)
- **CXR LATEST** [report_pdf/2026-04-13_XRay_X-Ray-Portable-Chest-AP.pdf](#) · [report_pdf/2026-04-06_XRay_X-RAY-Chest-AP.pdf](#) · [report_pdf/2026-04-03_XRay_X-Ray-Portable-Chest-AP.pdf](#) (35 chest films across the admission)
- **BIOMARKERS** [report_pdf/2026-03-24_Lab-Serol_NT-pro-B-type-Natriuretic-Peptide-NT-pro-BNP.pdf](#) (4089 pg/mL) · [report_pdf/2026-02-20_Lab-Serol_NT-pro-B-type-Natriuretic-Peptide-NT-pro-BNP.pdf](#) (1761 pg/mL)
- **DAYCARE** [report_pdf/2026-04-13_Summary-Daycare_Daycare-Discharge-Summary-Chemotherapy.pdf](#)

Master index: [MASTER_INDEX.md](#). All 188 reports as one file: [ALL_MD_REPORTS.md](#).

This handoff note was assembled from the patient's electronic record (188 reports, 20 Feb – 13 Apr 2026). All facts are sourced from the original PDFs cited above. Please cross-verify against the source PDFs before any clinical decision.